



• EPSDT

AETHERA • SPECIALTY BILLING SERIES

Pediatrics Billing Solutions

Maximizing Revenue Across Office E/M, Well-Child Visits, Vaccines, EPSDT, and
Pediatric Procedures

Prepared by: Aethera Healthcare Solutions • Lakeland, FL

Scope: CPT 99202–99215 99381–99397 90460–90474 69200–69910

J-codes (pediatric
vaccines)

Updated: 2026 Edition • aetherahealthcare.com • +1 (863) 694-0325



ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served
Across 25+ specialties

\$12M+

Revenue Recovered
For specialty practices nationwide

95%

Clean Claim Rate
vs. 84% industry avg

< 30d

Average AR
Days in accounts receivable



Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge. Direct line to your RCM lead — not a call queue.



Performance Targets, In Writing

96%+ net collection · < 5% denial · 95%+ coding accuracy · 24h posting · 48h submission · 65%+ appeal success



Zero Risk To Start

No setup fees. No long-term contracts. 90-day cancel anytime. Free 5-day audit of your current billing operation.



PEDIATRICS · MARKET CONTEXT

Why Pediatrics Billing Matters Now

Pediatrics is a high-volume, Medicaid-heavy primary-care specialty — 73M Americans under age 18, with 60%+ of pediatric claims tied to Medicaid/CHIP. Vaccine administration is the highest-volume revenue stream but carries the most coding-error risk.

\$98B



**US pediatric care spend
(2024)**

Includes well-child visits, vaccines, acute care, and chronic condition management.

73M



Americans under age 18

~22% of US population; ~90% insured via private or Medicaid/CHIP.

~ 9%



Pediatrics claim denial rate

Below specialty average, but vaccine coding errors and Medicaid eligibility lapses drive avoidable revenue loss.

\$3,500+

**Avg annual vaccine revenue /
provider**

Driven by vaccine administration fees + vaccine product reimbursement.

◆ **Net effect:** pediatric practices that master vaccine admin/product code matching, EPSDT documentation, and Medicaid eligibility re-verification can recover 6–10% of total revenue currently lost to denials and under-billing.



PEDIATRICS · OVERVIEW

The Four Pillars of Pediatrics Billing

Each pillar represents a distinct service family with unique coding, modifier, and documentation requirements.



Office E/M & Acute Visits

Office visits (99202–99215) for ear infections, URI, asthma, and acute pediatric complaints. 2021 E/M reform shifted documentation to MDM-based. Modifier 25 critical for same-day vaccine or procedure billing.

CPT 99202–99215 Modifier 25



Well-Child Visits & EPSDT Screening

Well-child (99381–99395), EPSDT screening (Medicaid), developmental screening (96110–96112). Age-based periodicity schedule drives visit cadence; documentation must match periodicity.

CPT 99381–99395 96110–96112



Vaccine Administration & Products

Vaccine admin (90460–90474), vaccine products (90630 influenza, 90686 HD flu, 90715 Tdap, 90371 RSV mAb). Admin + product code matching is the #1 denial driver. VFC documentation required for Medicaid.

CPT 90460–90474 90630 · 90715



Pediatric In-Office Procedures

Ear cerumen removal (69210), foreign body removal (69200), laceration repair (12001–12021), wart destruction (17110). Laterality and procedure documentation drive clean-claim rate.

CPT 69210 69200 · 12001–12021



PILLAR 01 · E/M + ACUTE CARE

Office Visits for Ear Infections, URI, Asthma & Acute Care

Pediatric office encounters cover acute complaints (otitis media, URI, asthma exacerbation, fever) and chronic condition follow-up (asthma, ADHD, obesity). The 2021 E/M coding reform shifted documentation to MDM-based or time-based. Modifier 25 is critical when billing an E/M on the same day as a vaccine, well-child visit, or in-office procedure.



E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215. Modifier 25 required if same-day vaccine or procedure.



Acute care documentation

Document onset, severity, treatment plan. Otitis media, URI, asthma, and gastroenteritis are the highest-volume pediatric acute complaints.



Chronic condition management

Asthma, ADHD, obesity, and anxiety management require care-plan documentation. Behavioral health integration increasingly common in pediatrics.



Modifier 25 is the single most-misused modifier in pediatric billing — appending to E/M when same-day vaccine is given without separate E/M documentation triggers automatic denial.

TOP CPT CODES

Office E/M Reimbursement

Code	Service	Avg \$
99202	New patient, low MDM	\$113
99203	New patient, moderate MDM	\$167
99213	Est. patient, low MDM	\$92
99214	Est. patient, moderate MDM	\$131
99215	Est. patient, high MDM	\$179
99091	Chronic care review, ≥30 min	\$35

99213 is the most common pediatric E/M. 99214 captures moderate-complexity acute/chronic management. 99091 supports chronic care review — under-utilized in pediatrics.



PILLAR 02 · WELL-CHILD + EPSDT

Well-Child Visits, EPSDT Screening & Developmental Surveillance

Well-child visits follow the AAP periodicity schedule (≥30 visits birth to age 21). Medicaid EPSDT (Early and Periodic Screening, Diagnostic, and Treatment) requires comprehensive documentation across 5 domains — incomplete documentation triggers automatic denial.

01

Well-Child Visits (New Patient)

99381 (< 1yr, \$185); 99382 (1-4 yrs, \$165); 99383 (5-11 yrs, \$185); 99384 (12-17 yrs, \$239). Includes comprehensive history, exam, age-appropriate screening, anticipatory guidance.

02

Well-Child Visits (Established)

99391 (< 1yr, \$182); 99392 (1-4 yrs, \$165); 99393 (5-11 yrs, \$185); 99394 (12-17 yrs, \$217). Annual preventive visits for established patients.

03

EPSDT Screening (Medicaid)

Comprehensive Medicaid screening covering 5 domains: comprehensive unclothed exam, immunization status, developmental assessment, vision/hearing screening, and anticipatory guidance. Documentation must address all 5.

04

Developmental Screening

96110 developmental screening (\$5); 96112 developmental testing (extended, \$13). Standardized tool required (e.g., M-CHAT, ASQ). Without documented tool, auto-denial.

✓ EPSDT Documentation Checklist

Medicaid requires: (1) comprehensive unclothed exam, (2) immunization status, (3) developmental assessment, (4) vision/hearing, (5) anticipatory guidance. **Missing any domain = full denial.**

📅 Periodicity Schedule

AAP schedule: 9 visits in first 2 years, annual visits ages 3-21. Deviations from schedule require medical necessity documentation.

✦ Same-Day E/M + Well-Child

When a problem-focused E/M is billed same day as a well-child visit, append **modifier 25** to the E/M. Without modifier 25, the problem E/M will be denied as bundled.

PILLAR 03 · VACCINES

Vaccine Administration, Products & VFC Documentation

Pediatric vaccine billing is the highest-volume revenue stream and the highest coding-error risk. Each vaccine requires BOTH an administration code (90460–90474) AND a vaccine product code (e.g., 90630, 90715). Mismatched admin/product code pairs are the #1 denial reason. Vaccines for Children (VFC) program requires VFC eligibility documentation for Medicaid patients.



Vaccine Administration

90460 first vaccine/component (\$20); 90461 each additional component (\$15). For patients <8 yrs when counseling provided. 90471 first vaccine (\$25); 90472 each additional (\$20). For patients ≥8 or no counseling.



Vaccine Products

90630 influenza (\$21); 90686 HD influenza (\$30); 90715 Tdap (\$48); 90371 RSV monoclonal antibody (\$1,500+); 90680 rotavirus (\$95); 90700 DTaP (\$25). Each product code must pair with correct admin code.



VFC Program

Vaccines for Children program: free vaccines for Medicaid/uninsured/underinsured/AI-AN patients. Practice bills ONLY administration fee — **never bill vaccine product**. VFC eligibility must be documented each visit.

VACCINE REVENUE SNAPSHOT

Key Vaccine Billing Stats

\$20 + \$15

Vaccine admin per dose
90460 first / 90461 addl

~ \$3,500/yr

Average annual vaccine revenue per provider

\$1,500+

RSV monoclonal antibody (90371)
per dose

24 %

Pediatric denials from admin/product mismatch

! RSV mAb (90371) is highest-revenue vaccine product

Single dose can exceed \$1,500. PA denial risk is highest in Q4 (RSV season). Aethera PA tracker documents eligibility (preterm, CLD, hemodynamically significant CHD) before submission.

⚠ Admin/product mismatch drives 24 % of all pediatric denials — automated code-pair validation at claim submission is the single highest-impact intervention.



PILLAR 04 · PEDIATRIC PROCEDURES

In-Office Procedures: Ear, Foreign Body, Laceration & Wart

Pediatric in-office procedures include cerumen removal, foreign body removal, laceration repair, and wart destruction. Laterality documentation (left/right) and procedure-specific documentation (size, depth, technique) drive clean-claim rate.

01

Ear Procedures

69210 cerumen removal, unilateral (\$55);
69209 cerumen removal, bilateral (\$85).
69200 foreign body removal, external auditory canal (\$85). Laterality (left/right) MUST be documented — bilateral billing requires modifier 50.

02

Laceration Repair

12001 simple repair, scalp/neck/axillae (\$135);
12002 simple, trunk (\$185); 12013 simple, face/ear/eyelid (\$245); 12014 intermediate (\$195). Code by anatomic site + complexity (simple/intermediate/complex).

\$55

Cerumen removal (69210) unilateral

\$85

Foreign body removal (69200)

03

Wart & Lesion Destruction

17110 destruction of benign lesions, up to 14 (\$135); 17111 15+ lesions (\$185). Wart destruction frequency limited by payer (typically 3-4 sessions/year per body region).

04

Misc Pediatric Procedures

69200 foreign body removal ear (\$85);
69910 otologic procedures (\$450+);
11200 removal skin tags, up to 15 (\$85);
10060 incision/drainage abscess, simple (\$155).

\$245

Laceration repair face (12013)

⚠️ Laterality documentation is the #1 procedure denial — every ear procedure (69200/69209/69210) requires explicit left/right documentation. Bilateral procedures require modifier 50 or specific bilateral CPT code.



PEDIATRICS • BILLING CODES

Top CPT/HCPCS Codes & 2026 Reimbursement

Most-billed codes across the four pediatrics pillars.

Code	Pillar	Service	Modifier	Avg \$	Notes
99202	E/M	New patient E/M, low MDM	—	\$113	Acute visit, new patient
99203	E/M	New patient E/M, moderate MDM	—	\$167	Comprehensive new patient workup
99213	E/M	Est. patient E/M, low MDM	—	\$92	Most common pediatric E/M
99214	E/M	Est. patient E/M, moderate MDM	—	\$131	Acute/chronic management
99381	WC	Well-child <1yr, new	—	\$185	First well-child visit
99382	WC	Well-child 1-4 yrs, new	—	\$165	Annual well-child
99392	WC	Well-child 1-4 yrs, est.	—	\$165	Established patient well-child
99393	WC	Well-child 5-11 yrs, est.	—	\$185	School-age well-child
90460	VX	Vaccine admin, first component	—	\$20	Per vaccine, <8 yrs w/ counseling
90461	VX	Vaccine admin, each addl component	—	\$15	Multi-component vaccines
90630	VX	Influenza vaccine	—	\$21	Seasonal flu, 6mo+
90715	VX	Tdap vaccine	—	\$48	Tetanus/diphtheria/pertussis
90210	90210	Scrub removal, unilateral	-50 (bilateral)	\$55	Document laterality

MODIFIER REFERENCE

Modifier 25 — significant, separately identifiable E/M same day as vaccine or procedure.

Modifier 50 — bilateral procedure. **Modifier 95** — synchronous telemedicine via interactive audio/video.

CODING RULES

Vaccine admin (90460–90474) — must pair with correct vaccine product code (90630, 90715, etc.). EPSDT (Medicaid) — requires 5-domain documentation. VFC — free vaccine for Medicaid; bill admin only.

PEDIATRICS • DENIAL PREVENTION

Top 8 Pediatrics Denial Reasons & How Aethera Prevents Each

Pediatrics denial rates average 9% — below specialty average, but vaccine coding errors and Medicaid eligibility lapses drive avoidable revenue loss. Each denied claim costs \$25–50 to rework.

RED
DENIAL
24%

Vaccine admin + product code mismatch

ROOT : 90460/90461 components do not match vaccine product code (e.g., 90460 with multi-component vaccine but only 1 90461 billed).

FIX Aethera vaccine code-pair rules engine validates admin/product pairing at claim submission.

RED
DENIAL
19%

EPSDT screening documentation incomplete

ROOT : Medicaid requires comprehensive EPSDT documentation across all 5 domains; missing any domain triggers full denial.

FIX Aethera EPSDT documentation checklist enforces 5-domain completeness before claim release.

AMBER
DENIAL
14%

Modifier 25 misuse

ROOT : E/M + vaccine/procedure billed same day without separate documentation supporting E/M significance.

FIX Aethera modifier 25 documentation checklist before claim release.

AMBER
DENIAL
11%

Well-child + problem E/M same day

ROOT : Billing both without modifier 25 on problem E/M; payer bundles the problem E/M into well-child.

FIX Aethera same-day E/M + well-child rules trigger modifier 25 prompt.

AMBER
DENIAL
9%

Vaccine VFC documentation missing

ROOT : Vaccines for Children program requires VFC eligibility documentation for Medicaid patients.

FIX Aethera VFC eligibility verification at check-in; auto-document in EHR.

GREEN
DENIAL
7%

Developmental screening without documented tool

ROOT : 96110 billed without documenting standardized screening tool used (M-CHAT, ASQ, etc.).

FIX Aethera developmental screening documentation template enforces tool name + score.

GREEN
DENIAL
6%

Ear procedure without laterality documented

ROOT : 69200/69209/69210 billed without explicit left/right documentation.

FIX Aethera procedure documentation template requires laterality field.

GREEN
DENIAL
5%

Insurance eligibility lapsed

ROOT : Pediatric Medicaid eligibility requires frequent re-verification; lapse between visits triggers denial.

FIX Aethera Medicaid eligibility re-verification workflow runs monthly + before each visit.

◆ **Cumulative impact:** Aethera reduces pediatrics denial rate from industry avg 9% to < 3% — recovering ~\$415K/year per \$3.5M pediatric practice.



CASE STUDY • INTEGRATED PEDIATRIC RCM

6-Pediatrician Practice — Annual Revenue Impact

A \$3.5M-revenue practice models the combined upside of vaccine coding accuracy, EPSDT documentation discipline, and Medicaid eligibility re-verification. Baseline: 6 pediatricians, 14,000 active patients (60% Medicaid), 25,000 vaccine doses/year.

INTERVENTION 01

Vaccine Coding Accuracy

- › 25,000 vaccine doses/year across 14,000 patients
- › Pre-Aethera vaccine coding error rate: 18%
- › Post-Aethera vaccine coding error rate: 3%
- › Recovered doses: 3,750/year
- › Avg vaccine admin + product revenue: \$49/dose

ANNUAL REVENUE

\$183,750

INTERVENTION 02

EPSDT + Well-Child Documentation

- › ~8,400 Medicaid well-child/EPST visits/year
- › Pre-Aethera EPSDT denial rate: 14%
- › Post-Aethera EPSDT denial rate: 4%
- › Recovered visits: 840/year
- › Avg EPSDT/well-child claim value: \$172

ANNUAL REVENUE

\$144,480

INTERVENTION 03

Medicaid Eligibility Re-Verification

- › 8,400 Medicaid patients (60% of panel)
- › Pre-Aethera eligibility lapse rate: 12%
- › Post-Aethera eligibility lapse rate: 3%
- › Recovered eligibility lapses: 756 patients
- › Avg annual Medicaid revenue per patient: \$112

ANNUAL REVENUE

\$84,672

COMBINED ANNUAL RECO¹

\$412,902

Implementation cost (year 1): ~ \$120K (Aethera RCM fees + vaccine workflow + EPSDT platform + eligibility re-verification). Year-1 net: \$292,902.

ROI

240%





PEDIATRICS · COMPLIANCE HOTSPOTS

Where OIG, Medicaid RAC, and Commercial Payers Audit First

Top compliance risks across the four pediatrics pillars. Quarterly self-audits recommended.

+ PILLAR E/M + Acute Care	✓ PILLAR Well-Child & EPSDT	+ PILLAR Vaccines	+ PILLAR Pediatric Procedures
RED Modifier 25 misuse with vaccines — appending modifier 25 to E/M when same-day vaccine does not represent significant, separately identifiable service; OIG Work Plan 2025-2026.	RED EPSDT documentation incomplete — billing EPSDT screening without all 5 required domains documented; Medicaid RAC active audit area.	RED Vaccine admin + product mismatch — billing 90460/90461 components that do not match vaccine product code; OIG Work Plan active area.	RED Ear procedure without laterality — billing 69200/69209/69210 without explicit left/right documentation; auto-denial trigger.
AMBER E/M level over-coding — billing 99214/99215 without documentation supporting moderate/high MDM; common audit finding in pediatric E/M.	AMBER Well-child + problem E/M same day without modifier 25 — billing both without modifier 25 on problem E/M; results in E/M denial.	AMBER VFC eligibility not documented — billing VFC vaccine administration without VFC eligibility documentation; Medicaid fraud risk.	AMBER Laceration repair documentation insufficient — billing 12001-12021 without documenting size, depth, complexity (simple/intermediate/complex).
GREEN Telehealth without modifier 95 — billing pediatric telemedicine visits without modifier 95 (synchronous audio/video); payer-specific requirements vary.	LOW Developmental screening without documented tool — billing 96110 without naming standardized tool (M-CHAT, ASQ) and recording score.	LOW Vaccine without National Drug Code (NDC) — billing vaccine product without NDC; commercial payer requirement for drug identification.	LOW Wart destruction frequency exceeded — billing 17110/17111 beyond payer frequency limits (typically 3-4 sessions/year per body region).



OIG Work Plan 2025-2026 explicitly lists pediatric vaccine admin/product code matching and EPSDT documentation as active audit targets. Medicaid RACs have recovered **>\$80M** from pediatric practices since 2021.



Three Actions to Capture Pediatrics Revenue

For pediatric practice administrators and revenue cycle directors leading the next 90 days.

01 ACTION 01 · VACCIN

Automate vaccine admin + product code matching

Deploy rules engine that validates 90460/90461 component counts against vaccine product code (90630, 90686, 90715, etc.) at claim submission. Auto-flag mismatched pairs for review before release.

↗ ~ \$185 K/yr
Per 25,000-dose pediatric practice

02 ACTION 02 ·

Enforce EPSDT documentation completeness for Medicaid

Build EHR template requiring all 5 EPSDT domains documented before claim release: comprehensive unclothed exam, immunization status, developmental assessment, vision/hearing screening, anticipatory guidance.

↗ ~ \$145 K/yr
Per 8,400-visit Medicaid pediatric practice

03 ACTION 03 · MEDICAID

Deploy Medicaid eligibility re-verification workflow

Run monthly Medicaid eligibility re-verification across active panel + point-of-service check at each visit. Flag eligibility lapses 7 days before they occur.

↗ ~ \$85 K/yr
Per 8,400-Medicaid-patient practice

Questions & Discussion

Optimizing Pediatrics Revenue Across E/M, Well-Child, Vaccines & Procedures

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